

1. Administrative & Study Identification

Full Study Title: AN INTERVENTIONAL OPEN-LABEL PHASE 1B/2 STUDY TO EVALUATE SAFETY, PHARMACOKINETICS, AND PRELIMINARY EFFICACY OF PF-08634404 AS MONOTHERAPY AND COMBINATION THERAPY IN ADULT PARTICIPANTS WITH UNRESECTABLE LOCALLY ADVANCED OR METASTATIC HEPATOCELLULAR CARCINOMA **Short Title/Acronym:** PF-08634404 Mono/Combo in Advanced Hepatocellular Carcinoma **Protocol Number:** C6461013 **NCT Number:** NCT07227012 **Sponsor Name:** Pfizer **Investigational Product:** PF-08634404 (PD-1 x VEGF Bispecific Antibody) alone and in combination with Ipilimumab (Yervoy) **Phase of Development:** Phase 1B/2 **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, pharmacokinetics, Phase 1b Dose Limiting Toxicities (DLTs), and preliminary efficacy (Objective Response Rate) of PF-08634404 as monotherapy and in combination with ipilimumab in adult participants with unresectable locally advanced or metastatic hepatocellular carcinoma (HCC). **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DOR), and the immunogenicity of PF-08634404.

2.2 Background & Rationale This study is being conducted to learn about the effects of PF-08634404 when given alone or with another antibody (ipilimumab) for the treatment of hepatocellular carcinoma (HCC) that is locally advanced (spread to nearby tissues) or has metastasized. There is a strong medical necessity to find effective treatments for patients who are not candidates for complete surgical or loco-regional therapies and have not received any prior whole-body systemic treatment.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Trial Status:** Not Yet Recruiting **Control Type:** Active Comparator / Single-arm (Monotherapy arm vs. Combination therapy arm) **Blinding:** Open-label **Randomization:** Randomized (in the Phase 2 portion of the study)

3.2 Planned Enrollment & Duration Target **SN\$:** 138 participants **Number of Sites:** Multiple global clinical trial sites **Estimated Study Start:** 01-Dec-2025 **Estimated Primary Completion:** 18-Oct-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. 18 years of age or older at screening. 2. Locally advanced or metastatic HCC with diagnosis confirmed by histology/cytology or clinically by AASLD criteria (for patients with cirrhosis). Participants without cirrhosis require histological confirmation of diagnosis. 3. Disease that is not amenable to curative surgical and/or locoregional therapies, or progressive disease after surgical and/or locoregional therapies. 4. At least 1 measurable (as defined by RECIST 1.1 per investigator) and untreated lesion. 5. Adequate hepatic, liver, and renal function; ECOG performance status 0 or 1; Child-Pugh Class A; No prior systemic therapy for HCC.

4.2 Exclusion Criteria 1. Moderate or severe ascites; History of hepatic encephalopathy. 2. Known active CNS lesions, including leptomeningeal metastasis, brainstem, meningeal, or spinal cord metastases or compression. 3. Clinically significant risk of hemorrhage or fistula. 4. Any history of another malignancy within 3 years. 5. History of allogeneic organ transplantation and allogeneic hematopoietic stem cell transplantation, or active autoimmune diseases requiring systemic treatment within the past 2 years. 6. Clinically significant cardiovascular disease within 6 months prior to the first dose. 7. Major surgery or severe trauma within 4 weeks prior to the first dose or planned major surgery during the study. 8. History of severe bleeding tendency or coagulation dysfunction. 9. History of severe ulcers, unhealed wounds, gastrointestinal perforation, abdominal fistula, gastrointestinal obstruction, intra-abdominal abscess, or acute gastrointestinal bleeding, including bleeding event due to esophageal and/or gastric varices, within 6 months prior to the first dose. 10. Participants with acute, chronic or symptomatic infections. 11. Participants with history of immunodeficiency.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Sequential dose levels of PF-08634404 and ipilimumab in Phase 1b (dose escalation), followed by selected optimized doses in Phase 2. **Route of Administration:** Intravenous (IV) infusions. **Duration of Treatment:** Cycles continue until disease progression, unacceptable toxicity, or subject withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures as allowed by protocol. **Prohibited:** Any prior or concurrent systemic therapy for HCC; other concurrent investigational treatments or biological products.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, RECIST Evaluation, eligibility confirmation
Baseline	Day 0	Randomization (Phase 2), First IV infusion dose
Interim	Every Cycle	Safety Labs, Efficacy Assessment (RECIST 1.1), PK and immunogenicity sampling
End of Study	EOT / Follow-up	Final Vital Signs, Exit Interview, Safety and survival follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definition: * Number of Participants With Adverse Events (characterized by type, frequency, severity per NCI CTCAE v5.0, timing, seriousness, and relationship to study intervention). * Phase 1b: Number of participants with Dose Limiting Toxicities (DLTs) during the DLT observation period. * Phase 2: Confirmed Overall Response Rate (ORR) defined as the proportion of participants with a confirmed Complete Response (CR) or Partial Response (PR). **Measurement Method:** RECIST 1.1 evaluated by the investigator, Central Laboratory Analysis for PK and Safety.

7.2 Secondary & Exploratory Endpoints * Phase 1b: Confirmed Objective Response Rate (ORR) * Progression-Free Survival (PFS) * Duration of Response (DOR) * Overall Survival (OS) * Immunogenicity of PF-08634404 in combination with ipilimumab

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring at clinical trial sites by trained medical staff during and after each infusion visit. **Serious Adverse Events (SAEs):** Evaluated strictly per protocol timelines (typically within 24 hours of site awareness). **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee (DMC) monitors safety and scientific integrity during dose escalation and optimization phases.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy assessments; Safety Set for all participants receiving at least one dose of the study drug. **Sample Size Justification:** Target $N=138$ is appropriately scaled to establish preliminary estimates of ORR, safety, and pharmacokinetics in both treatment arms. **Handling of Missing Data:** Standard multiple imputation or Last Observation Carried Forward (LOCF) for continuous variables;

censoring applied for time-to-event outcomes (PFS, OS, DOR) per statistical guidelines.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits to ensure subject safety and protocol adherence. **Audit Trail:** Robust Electronic Data Capture (EDC) system featuring eCRF locking, query management, and full audit trail traceability.

11. Study Identifiers & Governance

Primary ID: C6461013 **Registry Number:** NCT07227012 **Trial Status:** Not Yet Recruiting **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study to Learn About Study Medicine Called PF-08634404 as a Single Treatment and Combination Treatment in Adult Participants With a Liver Cancer Called Hepatocellular Carcinoma, That is Too Advanced to be Removed by Surgery and May Have Spread to Other Parts of the Body. **Official Regulatory Title:** AN INTERVENTIONAL OPEN-LABEL PHASE 1B/2 STUDY TO EVALUATE SAFETY, PHARMACOKINETICS, AND PRELIMINARY EFFICACY OF PF-08634404 AS MONOTHERAPY AND COMBINATION THERAPY IN ADULT PARTICIPANTS WITH UNRESECTABLE LOCALLY ADVANCED OR METASTATIC HEPATOCELLULAR CARCINOMA

12. Clinical Framework (Study Specific)

Primary Condition: Hepatocellular Carcinoma (HCC) **Disease Areas:** Carcinoma, Hepatocellular, Hepatocellular Cancer, Hepatocellular Carcinoma, Unresectable Hepatocellular Carcinoma, Liver Neoplasms, Advanced Hepatocellular Carcinoma, Metastatic Hepatocellular Carcinoma **Diagnosis Threshold:** Locally advanced or metastatic HCC confirmed by histology/cytology or clinically by AASLD criteria **Medical Methodology:** Systemic target/immunotherapy (PD-1 x VEGF Bispecific Antibody / CTLA-4 inhibitor) **Optimization Target:** Objective Response Rate (ORR) and tolerable dose determination

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for Unresectable / Advanced HCC **Education Protocol (HCP):** Site initiation visits and sequential dose-level protocol training **Education Protocol (Patient):** Informed consent process, ongoing treatment education, and side-effect reporting procedures **Quality Audit Mechanism:** Independent DMC tracking & Sponsor-led adherence monitoring

14. Observation & Follow-Up Schedule

Total Duration: Until study completion (Estimated Oct 2028)

Onsite Visit Cadence: Regular intervals aligned with IV infusion cycles and RECIST 1.1 tumor assessment schedules **Remote**

Monitoring Frequency: Continuous monitoring of AEs and survival follow-up periods
Checklist Review Interval: Routine safety lab reviews before each new treatment cycle

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					